

Chapter 10 — Ethics and Informed Consent — Numbers, Gold Standards and Traps

IF YOU READ NOTHING ELSE

- **Four principles** (Beauchamp and Childress 1979): autonomy, beneficence, non-maleficence, justice
- **Principles equal in weight**, each *prima facie* binding
- **Salgo 1957**: coined "informed consent"
- **Canterbury v Spence 1972**: reasonable-patient disclosure standard
- **Schloendorff 1914**: examination under anaesthesia (EUA) ≠ consent to surgery
- **Consent elements**: problem + options (incl. none) → risks/benefits/uncertainty → check understanding → consent
- **Commonest missed element**: assessing patient understanding
- **Material risk**: proposed ≥1% cut-off, rarer if severe
- **Conscious competent patient may refuse life-saving care** (Quackenbush 1978)
- **Surrogate standard**: substituted judgement → then best interests

MEDICAL ETHICS

- **Hippocratic Oath**: beneficence, non-maleficence, confidentiality, accountability
- **Oath's urological line**: "I will not cut persons labouring under the stone"
- **Oath's gaps (Veatch)**: no autonomy, no justice
- **"Bioethics" coined**: Van Rensselaer Potter, 1970
- **Tuskegee disclosed**: early 1970s → research consent
- **Autonomy**: Kantian; grounds consent, confidentiality, no deceit; bounded by justice
- **Non-maleficence**: *primum non nocere*
- **Justice ≠ equality**: equal treatment can still be unjust
- **Four-Box (Jonsen)**: medical indications → beneficence/non-maleficence; preferences → autonomy; quality of life → three principles; context → justice
- **Equivocal risk-benefit** → defer to patient preference
- **Quality of life**: most ephemeral box; beware provider bias

INFORMED CONSENT

- **Legal root**: battery (1905–14) → negligence (Kansas 1960)
- **Mohr v Williams 1905**: consultation ≠ consent
- **Luka v Lowrie 1912**: implied consent only in emergency, incapable patient
- **Negligence test**: below reasonable-physician standard + proximate cause
- **Discuss options before recommending** → less undue influence
- **Disclosing uncertainty** → better understanding and trust
- **Understanding biases**: shortcuts, optimism bias, framing
- **Emergency**: consent presumed (Barnett v Bachrach 1943, unconscious patient)
- **Children**: no waiver unless immediate harm (Rogers v Sells 1936)
- **Minors <18 y**: parents consent; child participates/assents
- **Surrogate failing patient's wishes** → ethics board → court conservator
- **Consent forms**: often >12th-grade reading level
- **Most effective comprehension tool**: repeat-back (time-costly)
- **Decision aids**: ↑ knowledge, ↓ decisional conflict
- **Simplified forms alone**: mixed results
- **Shared decision making (SDM)**: ~40 models; only bidirectional model
- **Models**: paternalistic (info + recommendation) vs informed (info only) vs shared (+ patient values)

END OF LIFE AND RESEARCH

- **Medicare**: 16% of annual budget in last 2 months of life
- **Uniform Health-Care Decisions Act (2006)**: futility judged by providers

- **Patient Self-Determination Act** (in force 1991): right to advance directive
- **Advance preferences → DNR (do-not-resuscitate) order**: <half
- **Research consent**: 45 CFR part 46 — "legally effective informed consent"
- **Waiver**: unlinked data, secondary data, or IRB (institutional review board): ≤ minimal risk + impracticable
- **>Minimal risk** → disclose compensation and injury treatment
- **Vulnerable groups (Subparts B–D)**: children, pregnant women/fetuses, disabled, prisoners

CLASSIC TRAPS

- **Trap**: Canterbury coined "informed consent" — **Actually**: Salgo 1957
- **Trap**: reasonable-physician disclosure — **Actually**: reasonable patient (Canterbury)
- **Trap**: autonomy trumps all — **Actually**: equal weight; bounded by justice
- **Trap**: justice = equality — **Actually**: equal treatment may be unjust
- **Trap**: emergency overrides conscious refusal — **Actually**: conscious patient may refuse
- **Trap**: family decides by own values — **Actually**: substituted judgement first
- **Trap**: reading the form ensures understanding — **Actually**: no better comprehension
- **Trap**: disclosing uncertainty erodes trust — **Actually**: increases trust
- **Trap**: consenting to EUA covers surgery — **Actually**: no (Schloendorff)
- **Trap**: quality-of-life box = autonomy only — **Actually**: autonomy, beneficence, non-maleficence